Patient-Direct Splint Billing: Legal Review Brief

Date: 2026-10-07
Status: Proposed option; not approved for launch
Purpose: Give healthcare counsel the facts and questions needed to assess the proposed billing and clinic-compensation structure. This document is not a legal conclusion.

Proposed Arrangement

Splint Factory is considering patient-direct payment as an alternative to monthly clinic invoicing.

1. A clinic evaluates and measures the patient, selects the clinically appropriate splint design, generates and locally fabricates the splint (using Splint Factory's system and an onsite 3D printer), fits and adjusts it, and documents the result.
2. The clinic retains clinical judgment and accepts responsibility and associated professional liability for design selection and proper fit.
3. Splint Factory provides the design-generation and workflow software, coordinates the patient checkout and transaction records, and is currently expected to be the merchant receiving payment.
4. The patient pays Splint Factory through hosted checkout, normally during the visit. Payment never gates clinical care, fabrication, or fitting; a patient may pay later.
5. Splint Factory pays the clinic for its measurement, design-selection, fabrication, fitting, adjustment, and associated responsibility. The original concept was a percentage of each patient charge; the preferred risk-reduction option is a fixed, fair-market-value fee for a defined service bundle.
6. Before payment, the patient is clearly told that Splint Factory is receiving the charge and that the clinic receives compensation for the services listed above. The disclosure is recorded.
7. The arrangement will not require exclusivity, minimum volume, referrals, or selection of a splint that is not clinically appropriate.

The initial market is Washington. Participating professionals are expected to include occupational therapists and certified hand therapists, but clinic ownership, practitioner licenses, employment relationships, and any physician involvement may vary.

Assumptions Requiring Confirmation

- The contemplated pilot is self-pay. Splint Factory would not submit insurance, Medicare, or Medicaid claims.
- We have not decided whether clinics or patients may submit receipts for reimbursement, use HSA/FSA funds, or bill separately for related professional services.
- We have not decided whether patients enrolled in federal healthcare programs may participate on a strictly self-pay basis.
- The final merchant/seller, allocation of the total charge between product and services, clinic fee amount, refund allocation, tax treatment, and contract terms are not yet fixed.
- The intended clinic liability allocation does not eliminate any nondelegable duty or product liability Splint Factory may have; counsel should define an enforceable allocation and required insurance.

Principal Legal Concerns

Washington fee splitting and professional conduct

- RCW 19.68.010 prohibits specified Washington licensees from receiving, and others from paying, a rebate, commission, unearned discount, profit, or other consideration connected with patient referrals or the furnishing of care, treatment, medical supplies, or prescribed goods and services. Its written-disclosure provision is specific to certain ownership interests and alternative facilities; it may not create a general disclosure defense.
- RCW 19.68.020 treats referral compensation received by a covered licensee as unprofessional conduct.

- RCW 19.68.030 authorizes license consequences for fee division or valuable consideration connected with therapeutic services, orthopedic appliances, or medical supplies.
- RCW 19.68.040 states the chapter's intent that covered licensees receive compensation only for professional services actually rendered, subject to its partnership and employment language.
- RCW 18.130.180(20) and (24) identify a chapter 19.68 violation and certain vendor gratuities presenting a conflict of interest as unprofessional conduct for license holders under the Uniform Disciplinary Act.
- WAC 246-847-160(16) and (19) address promotion for personal gain of unnecessary devices or services and chapter 19.68 violations in occupational therapy. WAC 246-847-170(2), (7), and (8) requires professional accountability and support for clinical records through observations or objective measures.

The fact that the clinic performs substantial services supports a service-compensation rationale, but counsel should determine whether the proposed payment remains an impermissible fee split or referral payment, whether chapter 19.68 covers each anticipated license type, and whether paying a clinic entity rather than an individual changes the analysis.

Federal fraud-and-abuse laws

- The federal Anti-Kickback Statute, 42 U.S.C. Section 1320a-7b(b), prohibits knowingly and willfully offering, paying, soliciting, or receiving remuneration to induce or reward referrals, orders, purchases, or recommendations involving items or services payable by a federal healthcare program.
- The personal-services safe harbor, 42 C.F.R. Section 1001.952(d), includes requirements such as a signed written agreement covering specified services, a term of at least one year, compensation methodology set in advance and consistent with fair market value, no determination based on federal-program referral volume or value, and commercially reasonable services. Counsel should determine whether this or any other safe harbor is applicable; the proposal should not assume safe-harbor protection.
- The physician self-referral law, 42 U.S.C. Section 1395nn, applies to certain physician financial relationships and Medicare designated health services. Its designated services include occupational therapy, durable medical equipment and supplies, and prosthetics and orthotics. Implementing rules appear in 42 C.F.R. Part 411, Subpart J.
- Medicare regulates durable medical equipment, prosthetics, orthotics, and supplies, including certain braces, under provisions including 42 C.F.R. Section 410.36, 42 C.F.R. Section 414.202, and DMEPOS supplier standards at 42 C.F.R. Section 424.57. Counsel should determine whether these custom finger splints and any associated services fall within a federally reimbursable category and whether self-pay participation by federal beneficiaries changes the analysis.

A private-pay design may reduce federal nexus, but it does not by itself resolve Washington law, and patient disclosure alone does not cure prohibited remuneration.

Proposed Risk Controls for Counsel Review

- Written agreement defining each clinic service, professional responsibility, clinical independence, documentation, liability, insurance, refunds, and compensation.
- Fixed fee for a defined, completed service bundle rather than a percentage of the patient charge, supported by a documented fair-market-value analysis.
- Payment only for services actually performed and documented, with no referral, exclusivity, minimum-volume, or federally reimbursed-business condition.
- Plain-language prepayment disclosure of the parties, financial relationship, clinic services, total price, and available patient choices, with evidence of disclosure.
- Separate accounting of the product/software charge and clinic service fee if counsel recommends it.
- A written payer policy covering federal-program beneficiaries, reimbursement submissions, HSA/FSA use, receipts, and any separate clinic billing.
- Contractual indemnity and insurance requirements that match each party's actual role without attempting to waive nondelegable duties.

Questions for Counsel

1. Is the arrangement permissible under Washington fee-splitting and professional-conduct law for each expected clinic and practitioner type?
2. Does compensation for the documented service bundle avoid characterization as referral remuneration? Is a fixed per-completed-service fee acceptable, and what fair-market-value support is needed?
3. Should the clinic, Splint Factory, or another entity be merchant/seller, and should the patient receive separate product and professional-service charges?
4. What exact patient disclosure, freedom-of-choice language, consent, and retention evidence are required? Does a list of alternatives need to be provided?
5. May Medicare/Medicaid enrollees use the self-pay option? Must the parties prohibit claims or reimbursement requests, execute any election or notice, or follow DMEPOS supplier rules?
6. Could Stark apply through a physician owner, employee, referral, or clinic relationship, and if so, which exception and documentation are required?
7. Does the proposed clinic role and liability allocation fit occupational therapy scope and professional standards? What product, professional, and cyber insurance should each party carry?
8. What restrictions should apply to marketing, pricing, discounts, hardship waivers, refunds, HSA/FSA representations, taxes, receipts, and separate clinic billing?
9. Can counsel provide an approved compensation structure, required contract clauses, patient disclosure, and pilot eligibility policy before launch?